

MUST KNOW THINGS IN ER

- NO FLUID TO ANY CARDIAC PATIENT
- Inj. Tramadol should be the last resort for pain management, and should always be given
i Inj. Reglan / Inj. Ondem.
- Any head trauma → do NCCT brain
(if GCS < 8, any bleeding from nose/mouth/ear,
any H/o LOC.)
- Any suspected CVA → check PLANTAR response
→ measure BP
↓
do NCCT brain
↓

<u>white</u> hemorrhage	<u>black</u> ischemia / SOL
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- Patient unconscious → check CBG (if no H/o trauma)
- Suspected intestinal obstruction → Dtg. straight Xray abd.
↓
supine
- Suspected hollow viscus perforⁿ → Dtg. straight Xray abd.
↓
erect
(gm ↓ diaphragm)
- Never administer — 3% NaCl
— Inj. Streptokinase
- Try to Remember GCS

Chest pain

↓
where?

pt. can point to one finger

mostly d/t musculo-skeletal origin

(Ddx: Costochondritis)

Try: Tramadol } 1 amp each
Try: Reglan } IM stat

for Home: Tabs. Tramadol 50
- 1 tab BDPC x 7d.
(+ antacid/PPI)

vague / compressive /
retrosternal / radiating
to neck & left hand

↓
? cardio origin

then: SOB (+)
(exertional)

↓
ECG → changes (+)

Trop T (+)

↓
give loading dose
as mentioned before.

respi origin

↓
✓ SOB +/-
✓ ECG - (N)
✓ Trop T -
✓ fever +/-

↓
∴ pleuritic chest pain

↓
→ Nebulise + Duolin: Budecort

⇒ 1:1

↓
then check ECG, CXR

↓
provide medication accordingly.

• SOB + + +
• fever (-)
• ECG (N)

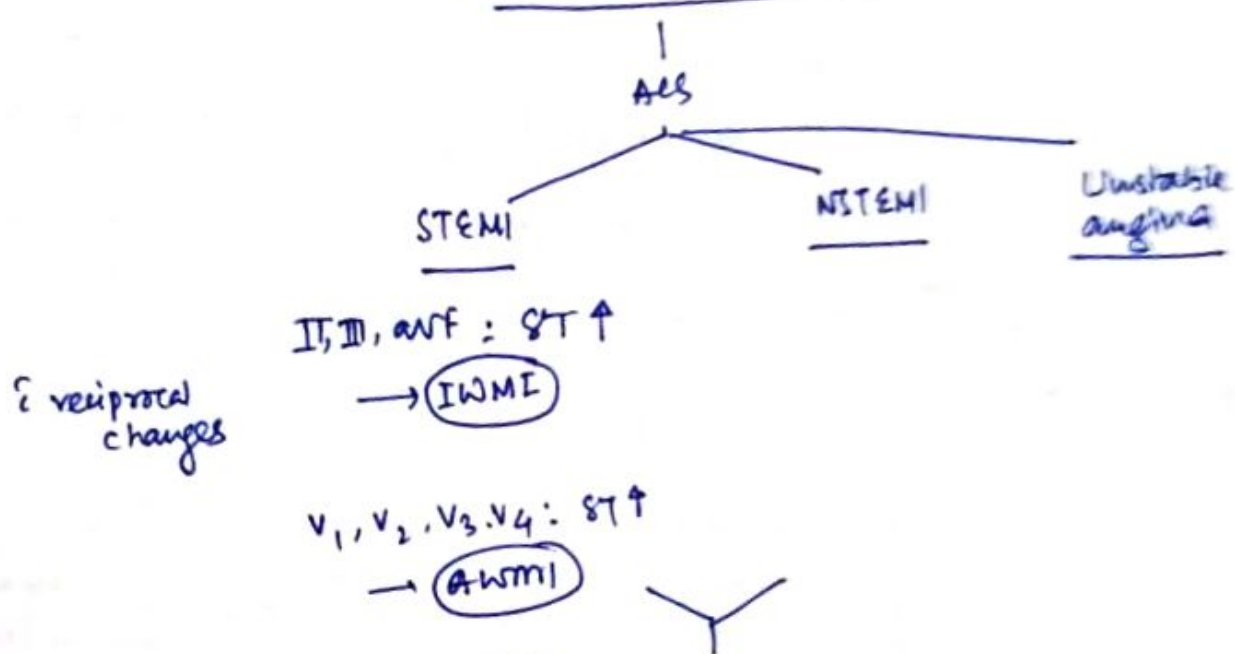
↓
CXR

{ History will guide }
? aortic dissection
? pulm. embolism

Always mention time & Date before giving device.

□ If patient comes with chest pain (Retrosternal) with radiation to left arm, shoulder, sweating →

- DO an ECG immediately.



Do Trop - T.

measure vitals : BP, PR, CG, SpO₂

If ST (↑) & Trop T positive

- T. Sorbitrate (5) 5/15 stat

Loading dose -

- T. Aspirin (75) x 4
- T. Clopidogrel (75) x 4
- T. Atorvastatin (40) x 2

Before giving loading dose
make sure that he has no peptic ulcer disease

Then Refer to Cardiology ER

(6)

Hemoptysis : Give Tranexa IV stat ✓

(coughing out Blood) — Chest X Ray PA view

— ↓

Refer to Chest OPD —

— Before SHOWING ANY CASE TO SENIOR
1st check vitals, take proper History
& examination

Acute Attack of Asthma

↓
pt will come c SOB → but $SpO_2 > 94\%$ in RA
↳ $\therefore T/T \rightarrow 0$ **SHIT**

Moist O_2

Salbutamol

Inj. Hydrocort
Lamp
IV

Ipratropium

Inj. Theophylline/Deniphylline
IV Inst

Nebulise c Dnolin: Budecort
1:1

Pt is SOB, no fever,

: check SpO₂
vitals.

Adv

- Moist O₂ inhalation

- Sig Deriphylin + Decadron
(1:1)

IV stat

if stable SpO₂ > 94%.

↓
CXR PA view

(check Pulse Rate)

✓ Nebulization

MDI is Duoalin 2 puffs stat

↓
2 puffs QDS

Duoalin:
Budecort.
(1:1).

(check SpO₂ in / out O₂)

✓ Hematemesis:

(Vomit out blood)

DCLD

✓ check CBG

✓ vitals: (BP) if low

✓ check if Hepatic
encephalopathy
present

✓ Fever is ↑ TLC

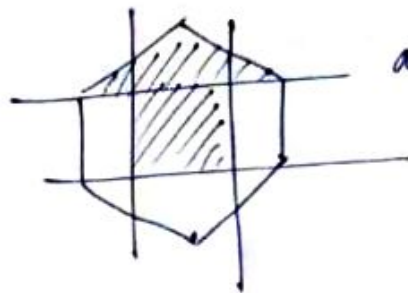
✓ check Plantar.

• Sig Tranexa IV stat
- 1 amp

IV fluid
is 500 ml
NS (Ringer's)

Spont. Bact. Peritonitis:

Pain abdomen.



1st line

any pain in marked area -

- ? gastritis
- ? acute cholecystitis.
- ? acute pancreatitis
- ? AMI (NEVER FORGET).
- ? referred appendicitis pain.

Inj. Diclin } 1 amp
Inj. Rantac } each IM stat.
(suspecting GI problem)

not resolved.

Inj. Pam 40 1 amp IM stat
(if still suspecting gastritis
w/ history)

still not resolved

always think
of AMI & do an ECG

[ANY PAIN from MANUBRIUM
to GROIN]



RLQ tenderness

- ✓ acute appendicitis
- ✓ ruptured ectopic
- ✓ tubal torsion
- ovarian

relate to history
& manage
accordingly.



- ✓ loin to groin pain
- ✓ back pain
(lumbar paraspinal)

think of Renal/urologic pathology

Inj. Voveran } 1 amp each
Inj. Rantac } IM stat.

[these pains, if assoc. w/ burning micturition,
dysuria]

UTI

- Symp. Atkars 10 ml in 1/2 glass water - BD x 7 d.
- Tabs. Nitrofurantoin 100 mg BD x 7-10 d
- Tabs. Urilpas 200 mg - TDS x 7-10 d.

check
Urgent Amylase,
lipase

Hemiparesis

(4)

- clo (L)/(R) sides
- weakness
- angle of mouth deviated
- slurring of speech
- Bladder, Bowel
- Plantar (R)
- (L)

elderly pt. disoriented

- check CBG
- send blood for $Na^+/K^+/Uy/Cr.$

urgent $\Rightarrow$

if possible; [Power
Tone
BP -
PR -
CBG -
Aor
NCT
Brain
 $\downarrow$

□ Fever + Chills + rigor :

- send blood for urgent cbc, MP + MPDA

□ pain d/t trauma

: Sij voveran 1M } stat
Sij Rantae 1M

□ Diarrhoea + fever / bloody Diarrhoea

check
Tongue,
Urine output

check BP, PR. send blood for $Na^+/K^+/Uy/Cr.$

IVF + Ciprofloxacin (100) X 2	Econorm Sachet, T. Racecadotril,
IVF + Metrogyl (200) X 2	
IVF + NS: RL	

any trauma / Head trauma pt / R.TA

C/O
cut injury

DOI
MOI
SOI

O/E:

any TT 0.5ml
IM stat

Adv: Refer to
ESOT (GS).

C/O Head injury.
d/t fall from height.

DOI - (Date of Injury)

MOI - mode of Injury

SOI - Site of Injury

O/E: GCS ✓

✓ BP

✓ PR

if active bleeding (+) →

NCCT Brain

↓ Adv

Refer to NSER

Guj. TT 0.5ml
IM stat

1st Refer to
ESOT (GS)
for bleeding
management

any ortho case

C/O trauma / pain in

DOI
MOI
SOI

O/E:

Tenderness +/-

Swelling +/-

Active Finger movement +/-

Distal Neuro vascular deficit +/-

Adv
Digital X-Ray of that
site of injury

AP / Lateral

if injury to
Hand / foot

then
AP / oblique

Glasgow Coma Scale

Response	Scale	Score
Eye Opening Response	Eyes open spontaneously	4 Points
	Eyes open to verbal command, speech, or shout	3 Points
	Eyes open to pain (not applied to face)	2 Points
	No eye opening	1 Point
Verbal Response	Oriented	5 Points
	Confused conversation, but able to answer questions	4 Points
	Inappropriate responses, words discernible	3 Points
	Incomprehensible sounds or speech	2 Points
	No verbal response	1 Point
Motor Response	Obeys commands for movement	6 Points
	Purposeful movement to painful stimulus	5 Points
	Withdraws from pain	4 Points
	Abnormal (spastic) flexion, decorticate posture	3 Points
	Extensor (rigid) response, decerebrate posture	2 Points
	No motor response	1 Point

Minor Brain Injury = 13-15 points; **Moderate Brain Injury** = 9-12 points; **Severe Brain Injury** = 3-8 points

GOVERNMENT OF WEST BENGAL
DEPARTMENT OF HEALTH & FAMILY WELFARE
RECORD OF EMERGENCY PATIENT
N.R.S MEDICAL COLLEGE & HOSPITAL
138, A.J.C BOSE ROAD, KOL-700014
(PH:(033) 2286-0103-08)

Patient Serial No: NRSM/PV2200030305		Date Time: 07-05-2022 8:28AM	Registraton No: NRSM/RG2200316932
Patient Name: NUR ISLAM MISTRI		Sex: Male	Age: 65 yrs 0 months 0 days
Disease:		Doctor: S.R.O.D	
Provisional Diagnosis: S		Referred From:	
Specify if it is a cause of Accident/ Suicide/Homicide :	How Injury Occurred :	Specify the place of injury Home/Farm/ Factory/Street/Others :	Whether injury occurred while at work Specify by Yes/No :
Complain 9:00pm C/O - Chest pain. radiating for the last 9 days.	Investigation ① ECG - 12 leads ② STrop T → positive. ECG → ST elevation in aVR, V ₁ -V ₄ , LBBB, Q LAD.	Advice STrop T → positive (+). ① Tab. Aspirin 75 4 tab stat ② Tab. Pan 40 x 1 tab stat ③ Tab. Aforvastatin 20 mg x 4 tab stat ④ Tab. clopidogrel 75 mg x 1 tab stat ⑤ Tab. sorbitrate 10 mg sublingual stat. Refer to cardiology ER Pt. to be transfused in trailer	
Vitals BP - 125/88 mm Hg PR - 112/min SpO ₂ - 98% CBG - 163 mg/dL Pedal edema (+) K/C/O - HTN.			

Signature of the Medical Officer

ADMISSION ALLOW
THROUGH ER SLIP

Name.- Mahila SK

Age.- 45

Sex.- F

Ward.- MSFS (Head Injury) under NS

Bk No.-

Date.- 4/5/22 → 5:43 pm

N.P. MCH.

10/11

Pt. gasping, trauma on head
SpO₂ - 92%, active seizure
present

[Signature]

< 200

~~200~~ 220, 230

251 - 300 → 8 units S/C. I.V.

300 - 400 → 8 units S/C. → I.V.

> 400 → H.I.

→ 10 units S/C. → 30 mins
↓
hypoglycemia

100

D5

80

20, 15

→ D 25

Date	Treatment
clo - Constipation → <u>bleeding</u> for the last 4 days.	P. Enema. <u>1 enema</u> Sy Monoplus / Piclin (Sodium Picosulfate) - 1Mf STD ~ 7pm x 7d
<u>vitals</u>	- <u>Smooth</u> / <u>Sucralbut</u> oral - To apply Rectally after defecation before ~
	Tds Ace + PCm Tds Pantoprazole - 40 - 15 OD NIV

N.O.O.E.M.
 Department of Community
 Medical Service
 N.K.B.M.C. & H.